



PERSONA

Tech-Hat Tasha

Scenario Author / Sim-Lab Tech

VRpatients Brand Foundation · Audit date 2026-05-22 · v2 · Locked palette
2026-05-25 · Run 1 of 6

Confidence tags [BEDROCK] evidence-locked [CRM-BACKED] CRM-supported [ASSUMPTION] needs human input

1. Identity

Tech-Hat Tasha is a community-college nursing or small-EMS-program sim coordinator who inherited the lab she runs, wears five other hats, and is the technical evaluator who decides whether VRpatients works on the ground. [ASSUMPTION] (handle proposed, pending Sylo approval).

2. Role Type

Heavy User [BEDROCK] and heavy Influencer [BEDROCK]. Sometimes the Buyer [BEDROCK] in small/community-college and small-EMS programs where one person does the evaluation, selection, and the purchase.

Touches both product components — the node-based authoring tool and the VR simulation itself. [BEDROCK]

3. Snapshot

- **Role.** Wears 2–6 hats simultaneously — faculty, lab manager, AV/IT troubleshooter, moulage, debriefer, equipment buyer. [BEDROCK]
- **Setting.** Community-college nursing program, small EMS / fire-EMS district, technical/career college, or K-12 / CTE health-science program. [CRM-BACKED] (K-12/CTE is a CRM-revealed segment the web research never named — 15+ entries; intent unconfirmed, see Q3.)
- **Tech comfort (bimodal).** CHSOS-track technologist (EMS/CNA/IT background) or faculty handed the sim lab (MSN/DNP nursing or paramedic instructor). [ASSUMPTION] (Sub-personas may need separating — Q9.)

- **Segment.** Nursing or EMS — but the CRM cannot reliably distinguish them: `ves_organization_type` is populated on only 13 of 1,016 closed deals. **[ASSUMPTION]**
- **Deal-size habitat.** \$5k–\$10k modal deal — 47% of closed-won deals sit here. **[CRM-BACKED]** Above \$10k, conversion gets harder (21% of losses in \$10k–\$25k vs 15% of wins). **[CRM-BACKED]**

4. Goals

- **Functional.** Build INACSL/NCJMM-aligned (nursing) or CoAEMSP-aligned (EMS) scenarios fast enough to keep up with cohort load. **[ASSUMPTION]**
- **Emotional.** Be the “wizard behind the curtain” — invisible competence; not get caught looking like a beginner. **[ASSUMPTION]**
- **Social.** Belong to SimHOSTS / SSH; hold CHSOS or CHSE credentials to be taken seriously. **[ASSUMPTION]**

5. Pains (ranked, most acute first)

1. **(Functional) “I’m a nurse, not a developer.”** Authoring tool learning curve eats hours she doesn’t have. **[CRM-BACKED]** (CRM: Greenfield CC, College Boreal, Walsh, Utah Tech. “Takes too much time to build” and “Hard to understand” exist as explicit `lost_reason` enum values in HubSpot.)
2. **(Emotional) Bought it, never used it.** Platform sits idle, no time to learn it. **[CRM-BACKED]** (Greenfield CC verbatim CRM lost-comment: “we have already wasted too much money without using it.” Pattern recurs at Medic Ambulance, Arrowhead EMS, Superior Ambulance, King County FPD#50.)
3. **(Financial) Renewal cliff for small programs.** \$1k–\$2k initial deals that became unrenewable when pricing went up; grant-funding dependence. **[CRM-BACKED]** (Mid-Minnesota EMS, John A Logan, Kishwaukee, SaveaLife, Excelsior Springs.)
4. **(Social) Champion-turnover orphan.** When she leaves, the platform leaves with her. **[CRM-BACKED]** (Lake Erie, Moraine Park, OCM BOCES, Laramie County, Medic Ambulance — 5+ CRM-documented losses to POC turnover.)

CRM bedrock: Wins are 62% **multi-contact**; losses are 53% **single-contact** (N=338 won / 678 lost). The lone-champion pattern correlates with losing. **[CRM-BACKED]**

6. What They've Tried Before & Why It Failed

- **Laerdal LLEAP / SimDesigner.** Manikin programming environment treated as too programming-heavy by non-developers. **[ASSUMPTION]**
- **vSim / Shadow Health (Wolters Kluwer).** Premade-only library; no custom-scenario depth. **[BEDROCK]** Tightly bundled into curricula at some institutions. **[CRM-BACKED]** (South College loss explicitly cited WK + SimX bundling.)
- **UbiSim / SimX / Intulect.** VR competitors actively winning. **[BEDROCK]** SimX named in 4 explicit CRM losses (Vocational Edu Board, Grand Canyon, City of Mt. Juliet, Sierra College); Intulect at UT Arlington (5-year contract). **[CRM-BACKED]** SimX wins on multi-user, pediatric, and WK-bundle narratives.

- **Doing nothing.** Constricting clinical placements (pediatrics, OB, psych) force the issue eventually. **[ASSUMPTION]**

7. Trigger Events

- **Lost grant funding** pressure-tests the budget conversation. **[CRM-BACKED]** (KenTenn, Lanier Tech, Cape Girardeau, Red Lodge, North Country.) Newly-awarded grants also drive activity.
- **Champion turnover** — incoming (new coordinator evaluates) or outgoing (existing buyer leaves, renewal dies). **[CRM-BACKED]**
- **Clinical-placement loss** in pediatrics, OB, or psych. Sim becomes substitute. **[ASSUMPTION]**
- **NCLEX-RN pass-rate drop / CoAEMSP site visit.** Dean walks in. **[ASSUMPTION]** (Zero CRM mentions of NCLEX / NGN / CoAEMSP in 95 free-text lost-reason comments — literature appears overweighted vs sales reality, see Q4.)
- **Conference exposure** at IMSh (January) or SimGHOSTS USA (summer). **[ASSUMPTION]**

8. Objections (top 3, categorized)

1. **“You don't have pediatrics.”** — Won't work for me. **[CRM-BACKED]** (Sierra College, Texas Tech UHSC El Paso, LSU Health Shreveport, City of Mt. Juliet.)

This is a narrative gap, not only a product gap. Sierra College went with SimX *after being told VRpatients had a pediatric patient.* The story isn't landing. Pediatric MVP (8–12-year-old male/female) is in active testing — see Q1.

2. **“You can only have one learner in the scenario at a time.”** — Won't work for me. **[CRM-BACKED]** (Vocational Edu Board, Northwest College, City of Mt. Juliet.) Sylo-confirmed: surfaces in demos and pre-demo qualifying — prevents some deals from starting at all.
3. **“We bought VR before, never really used it, can't justify another spend.”** — Don't need it / wrong timing. **[CRM-BACKED]** (Greenfield CC, Medic Ambulance, Arrowhead EMS, Superior Ambulance.) The persona's own pain becomes the next vendor's objection.

9. Watering Holes (named, specific)

- **Associations.** SimGHOSTS (simghosts.org + SimGHOSTS USA + SGX); SSH (ssih.org — owns CHSE/CHSE-A/CHSOS/CHSOS-A; runs IMSh); INACSL (owns the Healthcare Simulation Standards of Best Practice®); NLN; CoAEMSP (EMS-side; ACCREDITCON); ASPE. **[BEDROCK]**
- **Journals.** Clinical Simulation in Nursing (Elsevier, INACSL); Simulation in Healthcare (SSH); Advances in Simulation (BMC); Journal of Nursing Regulation (NCSBN). **[BEDROCK]**
- **Podcasts.** SimGeeks (Belk + Shablak); The Sim Cafe (Deb Tauber); Simulation Nation (Amy Cowperthwait); The Simulation Operative (Phillip Wortham); Bundle of Rays (Brad Chesham). **[BEDROCK]**
- **Content hubs.** HealthySimulation.com (Lance Baily); SIMZINE; Acadicus blog. **[BEDROCK]**
- **Influential people.** Penni Watts (INACSL Standards); Janice Palaganas (CMS); Mary Fey (CMS, debriefing); Jennifer McCarthy (EMS-sim standards); Matt Charnetski (nursing/EMS intersection); Erica Hinojosa (SimGHOSTS president); Melissa Milner (USF CAMLS). **[BEDROCK]**

- **Forums.** SimGHOSTS member forum (gated); HealthySimulation.com Community Groups (Scenario Design, Sim Tech Specialists); LinkedIn CHSOS/CHSE groups; r/nursing simulation threads; allnurses.com. **[ASSUMPTION]**

10. Voice of Customer

Pull quotes from CRM and adjacent sources. Tags indicate evidence level on whether the quote itself is verbatim from a VRpatients customer.

"We have already wasted too much money without using it."

Greenfield CC, CRM-verbatim lost-deal comment · [CRM-BACKED]

"Takes too much time to build. / Hard to understand."

Literal HubSpot lost_reason enum values · [BEDROCK]

"I'm a nurse, not a developer."

Inferred from pattern; not verbatim in CRM · [ASSUMPTION]

"More hats than they can comfortably wear."

Level3AV blog; not a VRpatients customer · [ASSUMPTION]

"Wizard behind the curtain."

Melissa Milner / USF CAMLS public interview; canonical in profession but not VRpatients-customer-verbatim · [ASSUMPTION]

Plausible 11pm Google search: "INACSL simulation design standard PDF" · "node-based scenario authoring nursing" · "how to write a simulation scenario template free." **[ASSUMPTION]** (Plausible based on what content ranks; not validated by search-volume data.)

11. Before / After

BEFORE VRpatients works

- New coordinator inherits the lab → 3–6 months figuring out existing tools → scenarios stale → utilization flat → dean cuts renewal.
- OR technologist sim tech spends 80–120 minutes per scenario → can't keep pace with cohorts → defaults to pre-built library only → loses the customization story.

AFTER VRpatients works

- Faculty-handed deploys first custom scenario in [X minutes — unmeasured **[ASSUMPTION]** — Q2], no code.
- 2nd internal champion onboarded within 90 days → platform doesn't depend on one person. **[CRM-BACKED]** (62% of wins are multi-contact; CS motion does not yet exist — Q8.)

- Scenarios map to INACSL Simulation Design or CoAEMSP capstone on a single downloadable PDF for audit time.
- [ASSUMPTION]**

The gap is time-to-value-without-becoming-a-programmer. VRpatients wins when it closes that gap visibly, and ships the second-champion handoff so a single departure doesn't kill the account.

12. Confidence Ledger

[BEDROCK]

- Persona identity: overworked, multi-hat USER who frequently buys in small programs.
- Watering holes: SimGHOSTS, SSH/IMSH, INACSL, HealthySimulation.com, SimGeeks.
- Top objection categories: specialty gaps; bought-but-not-used.

[CRM-BACKED]

- Lone-champion deals lose (62% multi-contact wins vs 53% single-contact losses).
- Slow-death pattern: 50% of losses take 180+ days vs 19% of wins.
- \$5k–\$10k modal deal; \$10k+ ceiling.
- Pediatric narrative gap (Sierra College).
- “Bought, never used” (Greenfield CC verbatim).
- K-12/CTE segment exists in funnel.
- SimX = most-named competitor in losses (4 explicit).

[ASSUMPTION]

- Actual minutes for time-to-deployed-scenario (Q2).
- NCLEX/NGN/CoAEMSP trigger reality (Q4).
- Two sub-persona split (Q9).
- Cybersickness as real objection (Q10).
- Verbatim customer language (Q16).
- “11pm Google search” phrasing.

Open Questions Routed to Team

Full traceable list in brand/personas/persona-audit-open-questions.md → “Tech-Hat Tasha — Open Questions” section. Summary by team:

- **Product.** Pediatric MVP ship date (Q1); authoring time measurement (Q2); multiplayer roadmap (Q6).
- **Sales.** K-12/CTE intent (Q3); NCLEX trigger (Q4); multiplayer counter-script (Q6); WK+SimX bundle map (Q7); sub-persona split (Q9); cybersickness reality (Q10); slow-death threshold (Q11); lost_reason taxonomy (Q12); hs_buying_role backfill (Q13); trial-stage hygiene (Q14); \$10k pricing ceiling (Q15).
- **Customer Success.** Bought-but-not-used frequency (Q5); second-champion implementation (Q8); VOC verbatim audit (Q16).
- **Leadership.** K-12/CTE strategy (Q3); kill-deal threshold (Q11); CRM hygiene calls (Q12/Q13); pricing tier structure (Q15).

Composite persona. No single named customer was used. Aggregate patterns only. Pre-questionnaire base draft — not for promotion to outputs/final-deliverables/ until the sales / CS / product / leadership questionnaire closes the [ASSUMPTION] gaps.